

Policy on Uniform Patient Care and Clinical Care Pathways

«Hospital Name»

Document control

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Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» ensures that patients with the same clinical condition and care needs receive the same quality of health care throughout the organisation. Care is provided uniformly across settings and ward categories and does not depend on whether the patient is paying or non-paying or on the source of payment.

The organisation uses patient identification processes, clinical practice guidance, clinical care pathways and multidisciplinary care planning to support uniform patient care.

Where telemedicine services are provided, they are delivered safely and securely based on written guidance.

1. Purpose

The purpose of this policy is to establish requirements for uniform patient care, patient identification, clinical practice guidance, clinical care pathways, multidisciplinary and multi-speciality care, and telemedicine where provided.

2. Scope

This policy applies to outpatient, day-care, inpatient and emergency care provided by «Hospital Name».

It applies across different categories of wards and settings of care and to patients irrespective of whether they are paying or non-paying or supported by a governmental or private insurance scheme.

This section applies where the organisation provides telemedicine services.

3. Policy standards

3.1 Uniform patient care

The organisation needs to implement mechanisms to ensure that care received in outpatient, day-care, inpatient and emergency settings is uniform.

The level of care provided to patients shall be the same throughout the organisation.

Patients with the same clinical condition and care needs shall receive the same quality of health care throughout the organisation, irrespective of the setting and category of ward and whether the patient is paying or non-paying or supported by a governmental or private insurance scheme or not.

Care is based on the clinical needs of the patient and not on the class or category of the patient.

Where the organisation has separate outpatient departments for different categories of patients, the methodology for care delivery shall be uniform in all outpatient departments.

3.2 Patient identification

The mechanism for identification of patients shall be uniform across the organisation.

For any care-related aspect, at least two identifiers shall be used.

One of the identifiers shall be the unique identification number generated at the time of registration.

The patient identification process shall be used in imaging, diagnostic and therapeutic procedures, blood transfusion, day care and nutrition or diet.

3.3 Evidence-based clinical practice guidelines and clinical protocols

The organisation implements evidence-based clinical practice guidelines and/or clinical protocols to guide uniform patient care.

Clinical practice guidelines brought out by national and international professional organisations may be used.

Standard treatment guidelines brought out by the Government of India are a good starting point.

In the absence of evidence-based clinical practice guidelines, or where adapting the clinical practice guidelines is not feasible, sound clinical practices shall guide the delivery of care.

For the definitions of evidence-based medicine and clinical practice guidelines, see the NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms.

3.4 Clinical care pathways

Clinical care pathways shall be multidisciplinary and based on evidence and/or best clinical practices.

They provide detailed guidance at various stages of care.

At a minimum, the organisation shall develop two clinical care pathways every year.

The clinical care pathways shall be followed consistently across all settings of care and shall be reviewed and updated annually.

3.5 Multidisciplinary and multi-speciality care

Whenever the patient's clinical condition warrants care from multiple disciplines and/or multiple specialties, a multidisciplinary and multi-speciality care plan shall be charted out based on established best clinical practices or clinical practice guidelines for the specific clinical condition.

An integrated care plan, including medical professional, nursing, nutritional and supportive care professionals, shall be developed and implemented appropriately by representatives of the various disciplines and specialities.

Examples of situations where multidisciplinary care may be required include care of a cancer patient determined by a multi-disciplinary tumour board, care of transplant patients, diabetes patients cared for by multiple specialists to prevent and manage end-organ damage, palliative or end-of-life care for terminally ill patients, and long-stay patients.

3.6 Telemedicine

This section applies where the organisation provides telemedicine services.

Whenever the telemedicine facility is provided, the organisation shall develop written guidance in consonance with prevailing laws and guidelines and implement the same.

The written guidance shall include protection of the patient's identity and confidentiality.

The limitations of information and communication technologies shall be explicitly addressed.

The organisation shall have a mechanism for appropriate data storage and retrieval.

The organisation shall have an MoU for its telemedicine services if outsourced.

4. Non-negotiable rules

- Do not provide different levels of care to patients with the same clinical condition and care needs because of payment status, source of payment, ward category or care setting.
- Do not use different care-delivery methods in separate outpatient departments for different categories of patients.
- Do not identify a patient for a care-related aspect using fewer than at least two identifiers.
- Do not omit the unique identification number generated at registration from the required patient identifiers.
- Do not use a non-uniform patient-identification process across the organisation.
- Do not leave care without evidence-based clinical practice guidelines or feasible adaptation of such guidelines without using sound clinical practices to guide delivery of care.
- Do not develop fewer than two clinical care pathways every year.
- Do not leave clinical care pathways without annual review and update.
- Do not omit multidisciplinary and multi-speciality care planning when the patient's clinical condition warrants care from multiple disciplines or specialties.
- Where telemedicine services are provided, do not provide them without written guidance developed in accordance with prevailing laws and guidelines.
- Where telemedicine services are provided, do not operate without protection of patient identity and confidentiality, explicit consideration of information and communication technology limitations, and a mechanism for appropriate data storage and retrieval.
- Where telemedicine services are outsourced, do not provide the outsourced telemedicine services without an MoU.

5. What we do

5.1 Provide uniform care

Care is provided uniformly across outpatient, day-care, inpatient and emergency settings.

Patients with the same clinical condition and care needs receive the same quality of health care throughout the organisation.

Care is based on clinical need and is not influenced by payment status, source of payment, ward category or care setting.

5.2 Identify patients consistently

The same patient-identification mechanism is used throughout the organisation.

At least two identifiers are used for any care-related aspect, with the registration-generated unique identification number as one identifier.

The process is used for imaging, diagnostic and therapeutic procedures, blood transfusion, day care and nutrition or diet.

5.3 Use clinical practice guidance

Evidence-based clinical practice guidelines and/or clinical protocols guide uniform patient care.

National and international professional organisation guidelines may be used.

Government of India standard treatment guidelines are a good starting point.

Where evidence-based guidelines are absent or cannot feasibly be adapted, sound clinical practices guide care.

The NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms is used for the definitions of evidence-based medicine and clinical practice guidelines.

5.4 Develop and maintain clinical care pathways

Clinical care pathways are multidisciplinary and based on evidence and/or best clinical practices.

At least two clinical care pathways are developed every year.

Clinical care pathways are followed consistently across all settings of care and reviewed and updated annually.

5.5 Plan multidisciplinary and multi-speciality care

When the patient's clinical condition warrants care from multiple disciplines or specialties, a multidisciplinary and multi-speciality care plan is prepared based on established best clinical practices or clinical practice guidelines.

An integrated plan covering medical, nursing, nutritional and supportive care is developed and implemented appropriately by representatives of the relevant disciplines and specialties.

5.6 Provide telemedicine safely where applicable

Where the organisation provides telemedicine services, written guidance is developed and implemented in accordance with prevailing laws and guidelines.

Patient identity and confidentiality are protected.

Limitations of information and communication technologies are addressed.

Appropriate data storage and retrieval are provided.

Where telemedicine services are outsourced, an MoU is maintained.

6. Governance and responsibility

Role	Responsibility
Management	Management ensures mechanisms for uniform patient care and supports implementation of the requirements in this policy.
Clinical leadership	Clinical leadership supports consistent care delivery, use of clinical practice guidance, development of clinical care pathways and multidisciplinary and multi-speciality care where clinically warranted.
Clinical and nursing personnel	Clinical and nursing personnel follow the applicable patient identification processes, clinical guidance, clinical care pathways and multidisciplinary care plans.
Telemedicine responsibility	Where telemedicine is provided, responsible personnel implement the written guidance and requirements for patient identity, confidentiality, technology limitations, data storage and retrieval and outsourced services.

7. Quality monitoring

Monitoring area	What is monitored
Uniform patient care	Same quality of care for patients with the same clinical condition and care needs across settings and ward categories
Patient identification	Uniform process and use of at least two identifiers, including the registration-generated unique identification number
Clinical guidance	Use of evidence-based clinical practice guidelines and/or clinical protocols, or sound clinical practices where stated
Clinical care pathways	Development of two clinical care pathways every year and annual review and update
Multidisciplinary care	Care plans when the patient's clinical condition warrants multiple disciplines or specialties
Telemedicine	Written guidance and required safeguards where telemedicine services are provided

8. Training and staff acknowledgement

Staff involved in patient care shall be familiar with the requirements for uniform care, patient identification, clinical practice guidance, clinical care pathways and multidisciplinary care applicable to their work.

Personnel providing telemedicine services, where applicable, shall be familiar with the written telemedicine guidance.

I have read the Policy on Uniform Patient Care and Clinical Care Pathways of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

9. Distribution

This policy shall be available to personnel involved in patient care and the implementation of the processes described in this policy.

Where telemedicine services are provided, the applicable written telemedicine guidance shall be available to the personnel responsible for those services.

10. Abbreviations

Abbreviation	Meaning
MoU	Memorandum of Understanding
ICMR	Indian Council of Medical Research
MoHFW	Ministry of Health and Family Welfare

11. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.1.a	Commitment*	Sections 3.1 and 5.1 address uniform care irrespective of setting, ward category, payment status or source of payment.
COP.1.b	CORE	Sections 3.2 and 5.2 address the uniform identification process, at least two identifiers and the registration-generated unique identification number.
COP.1.c	Commitment	Sections 3.3 and 5.3 address clinical practice guidelines, clinical protocols, sound

		clinical practices where required and reference to the NABH Guidebook Glossary of Terms.
COP.1.d	Excellence	Sections 3.4 and 5.4 address multidisciplinary clinical care pathways, two clinical care pathways every year, consistent use and annual review and update.
COP.1.e	Excellence	Sections 3.5 and 5.5 address clinically triggered multidisciplinary and multi-speciality care planning and integrated care.
COP.1.f	Commitment*	Sections 3.6 and 5.6 address telemedicine requirements conditionally where telemedicine services are provided.

12. Required Records/Evidence Checklist

Uniform patient care

- Written guidance or mechanisms showing uniform care across outpatient, day-care, inpatient and emergency settings.
- Care-delivery processes showing the same quality of health care for patients with the same clinical condition and care needs.
- Processes showing that care is not influenced by payment status, source of payment or ward category.
- Uniform care-delivery methodology used across separate outpatient departments where different categories of patients are served.

Patient identification

- Uniform patient-identification process used across the organisation.
- Patient identification records showing use of at least two identifiers for care-related aspects.
- Patient identification process showing use of the unique identification number generated at registration as one identifier.
- Patient identification process applied to imaging, diagnostic and therapeutic procedures, blood transfusion, day care and nutrition or diet.

Clinical practice guidance

- Evidence-based clinical practice guidelines and/or clinical protocols used to guide uniform patient care.

- National or international professional organisation guidelines where the organisation has chosen to use them.
- Government of India standard treatment guidelines where used as a starting point.
- Clinical practices used to guide care where evidence-based clinical practice guidelines are absent or cannot feasibly be adapted.
- Access to the NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms for the definitions of evidence-based medicine and clinical practice guidelines.

Clinical care pathways

- At least two clinical care pathways developed every year.
- Clinical care pathways showing multidisciplinary development and basis in evidence and/or best clinical practices.
- Clinical care pathways showing detailed guidance at various stages of care.
- Records showing clinical care pathways are followed across all settings of care.
- Records showing clinical care pathways are reviewed and updated annually.

Multidisciplinary and multi-speciality care

- Multidisciplinary and multi-speciality care plans for patients whose clinical condition warrants care from multiple disciplines or specialties.
- Integrated care plans covering medical, nursing, nutritional and supportive care where appropriate.
- Documentation showing participation by representatives of relevant disciplines and specialties.
- Care plans based on established best clinical practices or clinical practice guidelines for the specific clinical condition.

Telemedicine

- Written telemedicine guidance where the organisation provides telemedicine services.
- Telemedicine guidance aligned with prevailing laws and guidelines.
- Safeguards protecting patient identity and confidentiality.
- Written consideration of limitations of information and communication technologies.
- Mechanism for appropriate telemedicine data storage and retrieval.
- MoU for outsourced telemedicine services where applicable.

13. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.1.
2. Guidebook interpretation supplied for COP.1.a through COP.1.f.
3. National Accreditation Board for Hospitals and Healthcare Providers. NABH Guidebook to Accreditation Standards for Hospitals, 6th Edition, Glossary of Terms.

4. Telemedicine Practice Guidelines. Board of Governors, Medical Council of India; NITI Aayog; Ministry of Health and Family Welfare, Government of India. 2020.

Disclaimer

This policy reorganises the supplied COP.1 objective-element wording and Guidebook interpretation into plain-language policy format. The modal strength of the source has been preserved. Optional examples and mechanisms have not been converted into mandatory requirements. The exact requirements of at least two identifiers, two clinical care pathways every year, and annual review and update have been retained. Telemedicine requirements are conditional and apply where the organisation provides telemedicine services.